



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلي عبدالحليم سنهابى عزوز

المحافظة : الجيزة الحي / المدينة : بولاق الدكرور العنوان : ..

رقم البطاقة : 26708022301021 رقم التليفون : 01112849374 تاريخ تسجيل الإستمارة : 22-09-2026

العمر (تاريخ الميلاد) : 02-08-1967 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ليلي عبدالحليم سنهال عوز National ID: 26708022301021	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

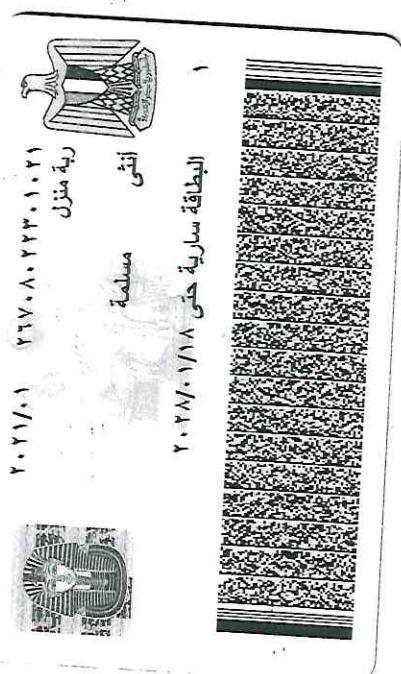
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



۱۔ ۲۔ ۳۔ ۴۔ ۵۔

Patient Name : - Mrs. Laila Abd Elhalim
 Referred by : - Prof. Magdy Mahfouze
 Date : - 06/05/26

High resolution US and Doppler study of the left breast

- A +/- 6.8 x 4.3 cm irregular shaped heterogeneously hypoechoic solid mass lesion at the 3 o'clock meridian of the left breast with scattered calcific foci of about 1-2 mm for each, no areas of cystic changes seen.
- Doppler study revealed prominent vascularity within ,showing high RI 0.81 .
- Enlarged lymph node seen in the region of the left axilla. appears with suspicious criteria (rounded of hypoechic texture) and eccentric hilum about 3 x 1.7 cm .

Conclusion:

- A suspicious lesion of the outer upper half of the left breast with left axillary suspicious lymphadenopathy (BI-RAD 5) , for biopsy

Thanks for your reference,

Yours truly

DR. Ashraf Rady

Consultant Radiologist

Patient Name

: - Mrs. Laila Abd Elhalim

Referred by

: - Prof. Magdy Mahfouze

Date

: - 06/05/26

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- A +/- 6.8 x 4.3 cm irregular shaped heterogeneously hypoechoic solid mass lesion at the 3 o'clock meridian of the left breast with scattered calcific foci of about 1-2 mm for each, no areas of cystic changes seen.
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Thanks for your reference,

Yours truly .

DR. Ashraf Rady

Consultant Radiologist



Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي القتر لانتونال - مصر معمل الباثولوجي



Patient Name	نبلى عبد العظيم سنهالى	Case No.	360291
Sex/Age	Female 59 Y	Patient No.	26295
Registered Date	24/8/2026 03:22:42 PM	Referred By Dr.	Interventional Radiology
Validation Date	22/9/2026 01:17:35 PM	Branch	Gustave Roussy
Reporting Date	22/9/2026 01:19:04 PM	Pathology No.	2723/26

Pathology Report

Clinical Data: Left breast UOQ mass (BIRADS 5) with left axillary lymph node.

Nature of Specimen:

- 1- Left breast mass, Tru-cut biopsy, collected on 24/08/2026 at 1:30 pm, Cold ischemia time: 20 seconds.
- 2- Left axillary lymph node, Tru-cut biopsy, collected on 24/08/2026 at 1:35 pm, Cold ischemia time: 20 seconds.

Gross Examination:

Two containers were received, designated as follow:

- 1- Left breast mass: Fragmented grey, white tissue cores, ranging in size between 0.2 & 0.8 cm, totally submitted.
- 2- Left axillary lymph node: One grey, white tissue core, measured 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- 1- Left breast mass:
 - Histologic type: Invasive duct carcinoma.
 - Histologic grade: 2 (differentiation 2, pleomorphism 2, mitosis 2).
 - DCIS: Absent.
 - Microcalcification: Absent.
 - Angioinvasion: Absent.
 - Perineural invasion: Absent.
 - Necrosis: Focal.

2- Left axillary lymph node:

- Histopathological examination reveals fibrotic cores showing positive metastatic deposits of tumor of breast origin. No nodal tissue in examined sections.

Results:

* IPT (All controls showed appropriate reactivity):

- 1- Left breast mass:
 - ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - الكادر 5 - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Miter El Qadima - Cairo, Egypt

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترنشنال - مصر - مصر لعمل الباثولوجي

Patient Name	نبلى عبد الحليم سنهالى	Case No.	360291
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- PR: Moderate nuclear reaction in 1% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

2- Left axillary lymph node:

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 10% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

Conclusion:

Left breast UOQ mass with left axillary lymph node, Tru-cut biopsies:

1- Left breast mass:

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.6), Her2/CEP17 ratio: (2.04)).
- Ki67%: 25%.

2- Left axillary lymph node:

- Consistent with fibrotic cores showing positive metastatic deposits of tumor of breast origin.
- No nodal tissue.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (4.05), Her2/CEP17 ratio: (2.1)).
- Ki67%: 25%.

****Scoring system: "ER, PR"**

خريطة رقم (4) - الطور الأول - مبنى رقم (987) - كور نيلز (نيلز) - مصر القومية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alisar El Qadema - Cairo, Egypt

Patient Name	ابلي عبد الحليم سنهلي	Case No.	360291
Sex/Age	Female 59 Y	Patient No.	26295
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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



عقبة رقم (4) - القدر الأول - مبنى رقم (987) - طر شمس النيل - مصر القوية - 5 مارس
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Almr El Qadema - Cairn, Egypt

Patient's Name:

Leila Abdelhalim Senhabi Azzouz

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast lump.

Findings:

Right breast

- Scattered fibroglandular densities. (ACR b)
- No masses, speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs. No pathological infra-clavicular nodes.

Left breast

- Scattered fibroglandular densities. (ACR b)
- Thickened left breast skin associated with underlying large infiltrative central breast mass measuring roughly (5.5 x 5.5 cm).
- Ultrasound demonstrated large infiltrative central breast mass measuring (5.5 x 5 cm) being more evident in the left UOQ contacting the overlying skin at 1 o'clock position .
- Thickened skin reaching 8 mm with coarsened trabecular and minimal subdermal edema.
- Infiltrated nipple.
- Multiple (more than three) pathological left axillary LNs are noted at level 1, 2 , one forming axillary soft tissue mass measuring (3.8 x 3.5 cm).

Conclusion:-

- **Right breast (BIRADS 1): Normal study.**
- **Left breast (BIRADS 5): Left highly suspicious breast mass associated with inflammatory process and ipsilateral heavy nodal affection .**

Prof.MONA ZAKY



Patient's Name:

Leila Abdelhalim Senhabi Azzouz

Date:

2026-09-13

Referred by:

Prof.Dr. no doctor

Hospital:

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Prof.MONA ZAKY